SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Capecitabine + rectal radiotherapy

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Locally advanced rectal cancer with concurrent radiotherapy · NCCP 00586 · version 3 · 7-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Capecitabine tablets help pelvic radiotherapy work against locally advanced rectal cancer. Your team explains how this course fits with surgery and any other chemotherapy.

Schedule
Take capecitabine twice daily on Days 1–5 of each 7-day cycle with radiotherapy, for a maximum of 5–6 cycles. Days 6–7 are not routine tablet days. Follow the team's written instructions if radiotherapy or tablets are interrupted.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Capecitabine
ORAL CHEMOTHERAPY

- ☐ Diarrhoea and dehydration
- ☐ Painful hands / feet
- ☐ Mouth soreness and reduced intake
- ☐ Low blood counts / serious infection
- ☐ Chest pain / heart problems
- ☐ Severe early toxicity, including DPD-related toxicity

Pelvic radiotherapy
LOCAL RADIATION TREATMENT

- ☐ Bowel irritation / diarrhoea
- ☐ Pelvic skin soreness
- ☐ Urinary symptoms
- ☐ Fatigue
- ☐ Possible lasting bowel / bladder effects
- ☐ Sexual function / fertility effects

Confirm DPD testing, tablet directions, missed-dose and stop-and-call instructions, kidney/liver and blood monitoring, interactions, fertility and contraception. Discuss radiotherapy-specific early and late effects and the separate radiation consent process.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00586 v3.

SACTCheck v0.84.21
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